TE-02 Carbon Monoxide / Cyanide Exposure

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 28, 2026

Protect responders first. Stage or request the appropriate fire/HazMat resources before entering an unsafe atmosphere. Do not become an additional victim.

Recognition and immediate field flow

Step	Required action
1. Safe scene	Use PPE and appropriate fire/HazMat support. Remove the patient from the exposure when safe. Do not enter a hazardous atmosphere without the required training and respiratory protection.
2. Decontaminate	For liquid or industrial cyanide contamination, remove contaminated clothing and decontaminate as directed before ambulance transport. Gas-only exposure without liquid contamination ordinarily does not require patient decontamination.
3. ABCs + oxygen	Assess airway, breathing, circulation, mental status, and perfusion. Give 100% oxygen regardless of conventional pulse-oximetry reading; support ventilation or airway management as indicated.
4. Assess toxicity	Look for altered mental status, dyspnea, weakness, nausea/vomiting, syncope, seizure, chest pain, shock, neurologic deficit, or respiratory failure.
5. Monitor + access	Obtain glucose, 12-lead ECG, IV/IO, and cardiac monitoring as indicated. Use pulse CO-oximetry when available, but do not delay care or rely on one field SpCO value to rule out poisoning.
6. Transport / alert	Rapidly transport significant or symptomatic exposure. Call Poison Help (1-800-222-1222) and notify the receiving facility early for severe exposure or high cyanide suspicion.

High cyanide suspicion

Consider cyanide poisoning: enclosed-space fire, known industrial cyanide exposure, or ingestion with severe altered mental status, seizure, hypotension/collapse, or respiratory failure. Continue 100% oxygen and supportive care while arranging immediate consultation and transport.

Provider-level actions

Level	Actions
EMT	Remove from exposure when safe; provide 100% oxygen and ventilation support; obtain glucose; monitor closely; and rapidly transport.
AEMT	All EMT care plus IV/IO access, 12-lead acquisition, and authorized treatment of glucose abnormality or shock.
Paramedic	All prior care plus cardiac and pulse CO-oximetry monitoring when available, Poison Help/Medical Control consultation, and early destination notification for severe exposure.

Carbon monoxide safeguards

Topic	Required action
Pulse oximetry	A normal conventional SpO_2 does not exclude carbon monoxide poisoning. Provide 100% oxygen on clinical suspicion even when a standard pulse-ox reading appears normal.
CO-oximetry	A field pulse CO-oximeter may support assessment when available. Do not delay treatment or transport for measurement, and do not use a single field SpCO value alone to rule out significant exposure.
High-risk features	Loss of consciousness, neurologic deficit, dysrhythmia/ischemia, severe acidosis, or pregnancy require early receiving-facility notification for hyperbaric-consultation consideration.
Vulnerable patients	Pregnant patients, children, older adults, and patients with cardiac, pulmonary, or hematologic disease may have serious symptoms at lower measured CO levels.

Cyanide safeguards

Topic	Required action
Contamination	Do not place a patient with liquid or industrial cyanide contamination in the ambulance before indicated decontamination and HazMat direction. Avoid direct contact with contaminated clothing, skin, or emesis.
Antidote availability	A cyanide antidote is not stocked on Claiborne ground units. Do not delay supportive care, Poison Help/Medical Control consultation, or transport while seeking an unavailable antidote.
Clinical deterioration	Escalate care for collapse, seizure, coma, respiratory failure, hypotension, dysrhythmia, or cardiac arrest. Use the applicable airway, shock, seizure, and cardiac-arrest pathways.

Consultation and handoff

- Call Poison Help at 1-800-222-1222 for significant suspected CO or cyanide exposure; use Medical Control for severe exposure or care outside standing orders.

- Communicate source, route, estimated duration, enclosed-space or combustion history, symptoms and progression, decontamination performed, oxygen/ventilatory support, ECG/CO-oximetry findings when available, treatments, and response.

- Early destination notification is essential for severe CO exposure, high cyanide suspicion, pregnancy, persistent neurologic symptoms, cardiac involvement, or need for advanced toxicology or hyperbaric consultation.

Critical reminder: 100% oxygen, airway/ventilation support, safe decontamination when indicated, Poison Help consultation, and rapid transport take priority. Do not allow a normal conventional SpO_2 or an unavailable antidote to delay care.

Linked pathways

- Use the age-appropriate airway, altered mental status, shock, seizure, and cardiac-arrest protocol as indicated.

- This direct source PDF replaces the prior imported sheet and matches Claiborne-approved TE-02 app content reconciled October 28, 2026.